

Venus K-Glow Boosters

Doctor Training Knowledge Evaluation - Candidate Question Paper | Version A

Assessment code	VA-KGLOW-A-2026.08
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Candidate and assessment details

Doctor full name		Employee ID	
Branch / city		Assessment date	
Trainer		Examiner	
Start time		Finish time	

Assessment structure and instructions

Section	Format	Questions	Scoring
A	Core and applied MCQs	40	80 marks - 2 each
B	Written clinical questions	5	20 marks - 4 each
C	Advanced Distinction MCQs	5	Separate score /5

Time allowed: 90 minutes. This is a closed-book assessment unless the examiner states otherwise. Select one best answer for every MCQ and mark it on the separate candidate answer sheet. Written answers should be concise, specific and clinically relevant. Mobile phones, smart watches, notes, slides, manuals, AI tools and discussion with other candidates are not permitted. The Advanced Distinction score is separate and does not determine the basic pass result.

Section A - Core and Applied MCQs (Questions 1-40 | 80 marks)

1. A client arrives asking specifically for the "strongest imported booster" because it is trending online. According to the Venus K-Glow prescribing framework, what should the doctor do first?

- A. Select the most premium product so the client feels fully treated.
- B. Identify the dominant skin concern, select the correct K-Glow category, and then choose an approved product.
- C. Offer whichever approved product is currently in stock.
- D. Allow the client to choose the brand after reviewing the price sheet.

2. A 49-year-old patient has prominent lower-face jowling and asks for a K-Glow booster to lift the jawline. Her skin is mildly dry, but advanced laxity is the dominant concern. What is the best first-line direction?

- A. K-Colla Bounce alone because collagen treats all laxity.
- B. K-Glass Hydration because dehydration is also present.
- C. Energy-based tightening or surgical assessment, with K-Glow considered only for an additional skin-quality objective.
- D. K-PDRN Reset because it produces structural lifting.

3. A patient requests K-PDRN Reset over an area with active inflammatory acne, crusting and tenderness. What is the most appropriate decision?

- A. Proceed with shallow microneedling because PDRN reduces inflammation.
- B. Proceed only if the patient signs an additional consent.
- C. Delay the booster and manage the active inflammatory or infectious condition first.
- D. Change to K-Exo Glow on the same day because exosomes support recovery.

4. During consultation, a breastfeeding patient asks for a facial K-Glow booster. Which response best reflects the Venus policy taught in the training?

- A. Proceed with any non-injectable booster because it is topical.
- B. Proceed only with PDRN because it is DNA-derived.
- C. Do not prescribe the treatment during pregnancy or breastfeeding under the current Venus policy.
- D. Proceed if the patient has no previous allergy.

5. A product brochure describes a preparation as injectable, but the current Venus protocol lists it only for microneedling. What should the doctor do?

- A. Inject it because the manufacturer route is more important than the clinic protocol.
- B. Use whichever route produces faster results.
- C. Do not improvise; use a route that is supported by the product IFU and the current Venus-approved protocol.
- D. Dilute it with saline and inject superficially.

6. During a microneedling-assisted booster, the skin has developed uniform mild-to-moderate erythema without significant bleeding. The doctor considers increasing depth until there is aggressive bleeding. What is the correct action?

- A. Increase depth because bleeding confirms product penetration.
- B. Continue only until pinpoint bleeding is present across the entire face.
- C. Recognize that the intended endpoint has been reached and avoid unnecessary over-treatment.
- D. Stop and repeat the entire procedure one week later.

7. Which record best meets the minimum Venus documentation standard after a K-Glow treatment?

- A. Treatment category and doctor signature only.
- B. Product name, batch/expiry, quantity, route or device settings, treatment area, reaction/notes and doctor details.
- C. A photograph and the payment receipt only.
- D. Product brand and the patient's verbal confirmation that the treatment felt comfortable.

8. Which post-treatment finding should be treated as an urgent red flag rather than an expected temporary reaction?

- A. Small injection papules that are gradually settling.
- B. Mild lip sensitivity on the day of treatment.
- C. Increasing severe pain with blanching and delayed capillary refill.
- D. Temporary under-eye puffiness without pain or visual change.

9. Which patient is the best fit for K-Glass Hydration as the primary treatment?

- A. A patient seeking major cheek projection and correction of structural volume loss.
- B. A patient with dull, dehydrated, mildly crepey skin and fine dehydration lines.
- C. A patient with severe inflammatory acne and open lesions.
- D. A patient whose main concern is advanced jowling.

10. Which statement most accurately distinguishes a conventional hydration skin booster from bioremodelling as taught in the presentation?

- A. Every non-cross-linked HA booster is automatically a bioremodelling product.
- B. A skin booster mainly targets hydration and texture, whereas bioremodelling also aims to improve broader tissue firmness, elasticity and structure without acting as a traditional filler.
- C. Bioremodelling is simply a lower-dose dermal filler technique.
- D. Skin boosters add volume, whereas bioremodelling only brightens the skin.

11. Which description matches Profilo in the K-Glass product table?

- A. 40 mg cross-linked HA in 1 mL designed for focal volume replacement.
- B. 64 mg non-cross-linked HA in 2 mL, combining high- and low-molecular-weight HA in a thermally stabilized hybrid complex.
- C. 68 mg cross-linked HA with succinic acid in a 2 mL gel filler.
- D. 20 mg PN in 2 mL for periocular injection.

12. Under the standard facial BAP protocol, how is a complete 2 mL syringe distributed?

- A. Five points total at 0.4 mL per point.
- B. Ten points total at 0.2 mL per point.
- C. Ten points total at 0.1 mL per point, leaving 1 mL for maintenance.
- D. Twenty points total at 0.1 mL per point.

13. A doctor marks the tragus BAP point directly at the inferior tragus. According to the presentation marking guide, what correction is required?

- A. Move the point approximately 1 cm anterior to the inferior tragus.
- B. Move the point 2 cm posterior to the tragus.
- C. Move the point to the midpoint between the tragus and lateral canthus.
- D. Keep the point at the inferior tragus because that is the standard landmark.

14. A patient asks to buy only half a Profilo facial treatment and return later for the remaining product. What is the correct Venus response?

- A. Use 1 mL now and refrigerate the remainder for the same patient.
- B. Use 1 mL now and share the remainder with another patient the same day.
- C. Explain that the standard facial protocol uses the complete 2 mL syringe for one patient, with no half syringe, sharing or leftover use.
- D. Dilute the 1 mL with saline so all BAP points can be treated.

15. Immediately after an HA booster injection, the patient develops disproportionate pain, blanching and a mottled pattern. What is the best immediate response?

- A. Apply a cold compress and allow the patient to leave with reassurance.
- B. Continue injecting slowly to complete the syringe and reassess afterward.
- C. Stop injecting, keep the patient in the clinic, assess colour/pain/capillary refill/vision, call the senior doctor and initiate the Venus HA vascular protocol.
- D. Massage only and review the patient after 24 hours.

16. A patient reports blurred vision and severe headache immediately after an injectable HA treatment. Which action has the highest priority?

- A. Observe for 30 minutes because visual symptoms are often anxiety-related.
- B. Give oral analgesia and arrange a routine next-day review.
- C. Treat this as an emergency and arrange urgent ophthalmology/hospital referral while activating the clinic escalation pathway.
- D. Inject additional HA to test whether the symptoms worsen.

17. Which statement best reflects the final Venus injectable technique message?

- A. A negative aspiration result proves that an injection is safe.
- B. Aspiration may be used according to clinic protocol, but it does not replace anatomy, correct plane, small controlled dosing, slow low-pressure injection and continuous observation.
- C. Aspiration should never be considered under any circumstance.
- D. Needle length alone determines whether vascular compromise can occur.

18. Which presentation best fits K-Colla Bounce?

- A. Early laxity, crepey texture, fine lines and reduced skin resilience without major structural volume loss.
- B. A deep static fold that requires immediate projection.
- C. An active infected acne cyst.
- D. A large tear-trough hollow requiring structural assessment.

19. Which statement about collagen types matches the training?

- A. Type I mainly provides finer early repair support, while Type III provides most tensile strength.
- B. Type I supports firmness and strength, while Type III provides finer structural support and contributes to early repair.
- C. Type I and Type III are interchangeable marketing terms with no structural distinction.
- D. Recombinant collagen is harvested directly from the patient immediately before treatment.

20. What is the Venus-approved protocol for SeduSkin Hylagen3 in the current K-Colla category?

- A. Inject the 5 mL vial using BAP points.
- B. Use the complete 5 mL vial with microneedling; do not inject it.
- C. Use half the vial and store the remainder.
- D. Apply it only after lip microneedling.

21. A doctor argues that every collagen product should be microneedled because K-Colla is a microneedling category. Which response is most accurate?

- A. Correct; ingredients determine that every collagen product can be microneedled.
- B. Incorrect; route is product-specific. Rejuccoll is currently microneedled at Venus, while Karisma has its own injection technique and should not be treated as the same product.
- C. Incorrect; all collagen products should instead be injected through the Profhilo BAP points.
- D. Correct, provided the depth is at least 2.5 mm.

22. A patient with a darker, PIH-prone skin type is receiving a microneedling-assisted booster. Which adjustment best follows the training?

- A. Increase depth and passes because darker skin requires more stimulation.
- B. Use conservative settings, avoid unnecessary trauma and stop at the intended erythema endpoint.
- C. Aim for aggressive bleeding to reduce the risk of pigmentation.
- D. Apply a strong acid immediately afterward to prevent PIH.

23. Which comparison between PN and PDRN is consistent with the training?

- A. PN consists of shorter fragments and is always more fluid than PDRN.
- B. PN generally has longer chains and greater viscoelastic support, while PDRN consists of shorter fragments with a repair/recovery emphasis.
- C. PDRN is a cross-linked HA gel and PN is a collagen peptide.
- D. PN and PDRN are identical terms and should be used interchangeably.

24. Which explanation of PDRN is most appropriate for a patient?

- A. It is fish oil that permanently replaces damaged DNA.
- B. It is a stem-cell treatment that becomes new skin cells.
- C. It is a purified DNA-fragment ingredient used to support repair and recovery; it is not filler, fish oil, stem cells or guaranteed permanent regeneration.
- D. It is a muscle-relaxing toxin used for pore control.

25. Which plan follows the current Venus K-PDRN protocol?

- A. Inject Lapuroon facially because injectable delivery is always more effective.
- B. Microneedle the approved PDRN product using a controlled endpoint, or apply topically after RF microneedling where protocol allows; do not mix it with PRP, saline or another booster unless specifically approved.
- C. Mix PDRN with PRP to increase growth-factor concentration.
- D. Dilute PDRN with saline so it covers a larger area.

26. A patient reports a previous generalized allergic reaction after an unknown "salmon DNA" injection. She now requests K-PDRN Reset. What is the safest next step?

- A. Proceed because a previous reaction proves the product was active.
- B. Proceed with half the dose after oral antihistamine.
- C. Do not proceed routinely; clarify the prior reaction, ask specifically about fish/salmon/seafood allergy and obtain senior assessment before any decision.
- D. Change the product brand but keep the same treatment category without further assessment.

27. A patient purchased Rejuran Healer, but an assistant brings a syringe of Lapuroon Super into the room. What should the doctor do?

- A. Use it because both fall under K-PDRN Reset.
- B. Use it if the volume is similar and document the change afterward.
- C. Stop and verify the prescribed and purchased product, variant, quantity, batch and expiry before treatment; do not substitute without an authorized clinical and commercial decision.
- D. Mix the two products so the patient receives benefits of both.

28. Which description of exosomes is most accurate within the training framework?

- A. Living stem cells that permanently engraft in the skin.
- B. Cell-free extracellular vesicles that carry signalling cargo such as proteins, lipids, growth-related molecules and RNA.
- C. Cross-linked HA particles used for volume replacement.
- D. Botulinum toxin fragments that relax facial muscles.

29. Which product-source and preparation pairing matches the current K-Exo training?

- A. Exoxe - human amniotic-fluid stem-cell source; reconstitute the lyophilized vial with the supplied diluent for the skin protocol.
- B. Exoxe - salmon DNA source; inject without reconstitution.
- C. ASCE+ - bovine collagen source; dilute with PRP.
- D. ASCE+ - botulinum peptide source; use as intramuscular toxin.

30. Which statement should a Venus doctor avoid when counselling a K-Exo Glow patient?

- A. The aim is recovery support and gradual skin-quality improvement.
- B. Product source, manufacturing, storage and intended route affect quality.
- C. This is stem-cell therapy that repairs DNA and permanently reverses ageing.
- D. The evidence is promising, but products and outcomes vary.

31. A patient requests K-Lumi Eyes for dark circles. Examination shows marked tear-trough hollowing, fat prolapse and chronic malar edema after previous filler. What is the best response?

- A. Proceed with large PN boluses because the product will replace the lost volume.
- B. Explain that K-Lumi is not the correct primary solution, assess the structural and edema causes, and delay or redirect treatment.
- C. Microneedle aggressively because microneedling avoids all under-eye risk.
- D. Inject hyaluronidase into the PN product before treatment.

32. Which under-eye injection approach best reflects the K-Lumi safety slide?

- A. Deep orbital placement at 90 degrees with large boluses.
- B. Superficial placement in the papillary dermis using a conservative product-specific technique, approximately 10-20 degrees, while avoiding large boluses and the lash line.
- C. Subperiosteal placement using the entire needle length.
- D. Intramuscular injection to reduce orbicularis activity.

33. After a pure PN under-eye booster, the patient develops severe swelling and eye pain. Which statement is correct?

- A. Hyaluronidase will dissolve the PN product, so no further assessment is required.
- B. Eye pain or severe swelling requires urgent medical review, and hyaluronidase does not dissolve pure PN/PDRN products.
- C. This is always normal for 7-10 days.
- D. Massage the area forcefully until the swelling disappears.

34. Which expectation is appropriate for K-Lip Dew?

- A. Major lip enlargement and Russian-lip reshaping.
- B. Hydration, softness, improvement in fine lip lines and a natural sheen without a filler look.
- C. Correction of severe filler migration.
- D. Permanent pink pigmentation of the lips.

35. A patient had lip filler 5 days ago and still has bruising, swelling and a palpable nodule. She asks for K-Lip Dew today. What is the best decision?

- A. Proceed because a booster will smooth the nodule.
- B. Proceed with half the usual volume.
- C. Delay the booster, assess the existing filler and wait until the lips have settled; a practical interval is often about 2-4 weeks when no complication is present.
- D. Dissolve all filler and perform the booster in the same sitting without further assessment.

36. Which lip-booster practice is compliant with the training?

- A. Reuse the same stamp tip on the same patient at the next monthly session.
- B. Share an opened syringe between two patients if a new tip is used.
- C. Use a new sterile product-specific needle or tip for every treatment, document quantity, and do not share an opened syringe unless the IFU explicitly permits it.
- D. Store all opened lip boosters for later because they contain HA.

37. How does the objective of K-Tox Refine differ from conventional injectable Botulinum Toxin Type A treatment?

- A. K-Tox is intended mainly for clinically significant muscle paralysis.
- B. K-Tox focuses on pore appearance, sebum and superficial texture or fine lines rather than full muscle relaxation.
- C. K-Tox is a structural filler for volume loss.
- D. There is no difference; the terms are interchangeable.

38. What is the current Venus approach to Lumi Meso Tox under K-Tox Refine?

- A. Inject the product intramuscularly in standard Botox points.
- B. Use the complete 5 mL vial with the approved microneedling protocol; it may also be applied after RF microneedling where the clinic protocol permits.
- C. Use 0.5 mL and store the remainder for another patient.
- D. Mix it with injectable Botox to guarantee pore closure.

39. Which treatment-course entry should be written on the doctor sheet?

- A. Profhilo: one session only; all other boosters: one session only.
- B. Profhilo: two sessions at one-month intervals plus ongoing maintenance around every six months; other K-Glow boosters: three to four monthly sessions plus maintenance.
- C. All boosters: weekly sessions until visible peeling occurs.
- D. Profhilo: four sessions in one month; other boosters: no maintenance.

40. Which aftercare plan is most consistent with the universal K-Glow post-treatment guidance?

- A. Massage all treated areas frequently, resume retinol the same evening and use a hot shower to reduce bumps.
- B. Avoid touching, rubbing, makeup and excessive heat initially; use gentle skincare and sun protection; avoid strong actives for several days and other irritating procedures for about 1-2 weeks unless advised.
- C. Apply bleaching cream immediately to prevent pigmentation.
- D. Schedule an aggressive peel within 48 hours to improve penetration.

Section B - Written Clinical Questions (Questions 41-45 | 20 marks)

Answer each question on the separate written-answer sheet. Each question is worth 4 marks. Four clear clinical points are usually sufficient.

41. Protocol reconstruction - K-Glass facial BAP treatment

A suitable patient has selected a full-face Profhilo K-Glass treatment. Write the essential sequence from preparation to completion. Your answer should include four clinically important steps or controls.

42. Patient selection - active inflammatory skin before an event

A patient has active inflammatory acne with several excoriated lesions and wants K-PDRN Reset five days before a wedding. Explain your decision and immediate plan in four points.

43. Clinical modification - under-eye diagnosis

A patient requests K-Lumi Eyes for dark circles. Examination shows major tear-trough hollowing, chronic malar edema and recent under-eye filler. State four elements of your assessment and management plan.

44. Complication management - suspected HA vascular compromise

During K-Glass injection, the patient develops severe increasing pain, blanching and delayed capillary refill. Write the first four actions that should occur.

45. Integrated treatment planning and documentation

A bride has four months before her wedding. Her dominant concerns are dehydration, visible pores and mild under-eye crepiness. She asks to complete every booster in one visit. Propose a safe, realistic plan in four points.

Section C - Advanced Distinction MCQs (Questions 46-50 | Separate score /5)

These questions assess relevant knowledge beyond the exact presentation. They do not reduce the core score, but at least 4/5 is required for Pass with Distinction.

46. IBSA describes NAHYCO Hybrid Technology as which type of stabilization process?

- A. Chemical BDDE cross-linking of one molecular-weight HA fraction.
- B. A patented thermal process combining high- and low-molecular-weight HA without chemical cross-linking reagents, producing cooperative hybrid complexes.
- C. Freeze-drying HA with calcium hydroxylapatite particles.
- D. Enzymatic conversion of HA into PN.

47. Which pair best summarizes the two widely described biological mechanisms of PDRN in tissue repair literature?

- A. Muscarinic receptor blockade and calcium chelation.
- B. Adenosine A2A receptor activation and provision of nucleotides/nucleosides through the salvage pathway.
- C. Chemical cross-linking and acetylcholine inhibition.
- D. Melanin oxidation and collagen denaturation.

48. According to MISEV2023 nomenclature guidance, when is the term "exosome" most scientifically defensible?

- A. Whenever a product contains particles smaller than 200 nm.
- B. Whenever a supplier uses the word on the box.
- C. When endosomal/subcellular biogenesis has been demonstrated; otherwise a broader extracellular-vesicle term is generally preferred.
- D. Only when the vesicles are derived from adipose tissue.

49. Which statement reflects current U.S. FDA information on microneedling devices?

- A. Microneedling cartridges may be cleaned and reused on the same patient.
- B. FDA-authorized microneedling devices are approved to deliver cosmetics, drugs and PRP into the skin.
- C. A new cartridge should be used for each patient and each session, and FDA states that microneedling devices are not approved for delivery of cosmetics, topical medications, drugs or blood products into the skin.
- D. Pigment changes and infection are not recognized microneedling risks.

50. Which counselling statement is most defensible when discussing human-cell- or tissue-derived "regenerative" products in light of FDA safety communications?

- A. A human tissue source proves the product is safe and approved.
- B. Unapproved products may not have been reviewed for quality, safety, purity or potency, so disease-treatment or cure claims should be avoided.
- C. The word exosome means the product is exempt from drug and biologic regulation.
- D. A high particle count alone proves clinical efficacy.

End of Question Paper. Confirm that Questions 1-50 have been attempted before submitting your answer sheet.

Venus K-Glow Boosters

Candidate Answer Sheet | Version A

Assessment code	VA-KGLOW-A-2026.08
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Candidate details

Doctor full name		Employee ID	
Branch / city		Assessment date	
Start time		Submission time	

Section A - MCQ answer grid

Place one clear tick or circle in the selected option. Multiple marked answers will be treated as incorrect unless the examiner authorizes a correction.

Q	A	B	C	D	Q	A	B	C	D	Q	A	B	C	D	Q	A	B	C	D
1	☐	☐	☐	☐	11	☐	☐	☐	☐	21	☐	☐	☐	☐	31	☐	☐	☐	☐
2	☐	☐	☐	☐	12	☐	☐	☐	☐	22	☐	☐	☐	☐	32	☐	☐	☐	☐
3	☐	☐	☐	☐	13	☐	☐	☐	☐	23	☐	☐	☐	☐	33	☐	☐	☐	☐
4	☐	☐	☐	☐	14	☐	☐	☐	☐	24	☐	☐	☐	☐	34	☐	☐	☐	☐
5	☐	☐	☐	☐	15	☐	☐	☐	☐	25	☐	☐	☐	☐	35	☐	☐	☐	☐
6	☐	☐	☐	☐	16	☐	☐	☐	☐	26	☐	☐	☐	☐	36	☐	☐	☐	☐
7	☐	☐	☐	☐	17	☐	☐	☐	☐	27	☐	☐	☐	☐	37	☐	☐	☐	☐
8	☐	☐	☐	☐	18	☐	☐	☐	☐	28	☐	☐	☐	☐	38	☐	☐	☐	☐
9	☐	☐	☐	☐	19	☐	☐	☐	☐	29	☐	☐	☐	☐	39	☐	☐	☐	☐
10	☐	☐	☐	☐	20	☐	☐	☐	☐	30	☐	☐	☐	☐	40	☐	☐	☐	☐

Examiner use: Correct answers _____ / 40 MCQ marks _____ / 80

Section B - Question 41: Protocol reconstruction - K-Glass facial BAP treatment

A suitable patient has selected a full-face Profhilo K-Glass treatment. Write the essential sequence from preparation to completion. Your answer should include four clinically important steps or controls.

Examiner score: _____ / 4 Examiner initials: _____

Section B - Question 42: Patient selection - active inflammatory skin before an event

A patient has active inflammatory acne with several excoriated lesions and wants K-PDRN Reset five days before a wedding. Explain your decision and immediate plan in four points.

Examiner score: _____ / 4 Examiner initials: _____

Section B - Question 43: Clinical modification - under-eye diagnosis

A patient requests K-Lumi Eyes for dark circles. Examination shows major tear-trough hollowing, chronic malar edema and recent under-eye filler. State four elements of your assessment and management plan.

Examiner score: _____ / 4 Examiner initials: _____

Section B - Question 44: Complication management - suspected HA vascular compromise

During K-Glass injection, the patient develops severe increasing pain, blanching and delayed capillary refill. Write the first four actions that should occur.

Examiner score: _____ / 4 Examiner initials: _____

A bride has four months before her wedding. Her dominant concerns are dehydration, visible pores and mild under-eye crepiness. She asks to complete every booster in one visit. Propose a safe, realistic plan in four points.

Examiner score: _____ / 4 Examiner initials: _____

Section C - Advanced Distinction answer grid

Question	A	B	C	D
46	[]	[]	[]	[]
47	[]	[]	[]	[]
48	[]	[]	[]	[]
49	[]	[]	[]	[]
50	[]	[]	[]	[]

Advanced Knowledge Score: _____ / 5

Candidate submission declaration

I confirm that I completed this assessment independently and did not use unauthorized notes, devices, applications, AI tools or assistance. I confirm that this answer sheet represents my own work.

Candidate signature	_____	Date	_____
Examiner signature	_____	Date	_____

Venus K-Glow Boosters

Examiner Answer Key, Rationales and Marking Guide | Version A

Assessment code	VA-KGLOW-A-2026.08
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Scoring and result policy

Section	Scoring	Maximum
A - MCQs	2 marks for each correct answer; no negative marking	80
B - Written	Four predefined marking points per question	20
Core total	MCQ + written	100
Advanced	1 point each; recorded separately	5

Result categories: 90% or above plus at least 4/5 advanced = Pass with Distinction; 80% or above without distinction criteria = Pass; 70-79% = Retest Required after targeted review; below 70% = Retraining Required + Retest; incomplete or invalid work = Invalid Attempt.

Source-control rule used to construct this assessment

The uploaded 99-slide training presentation is the primary source for scored core questions. The English transcript is used to capture verbal teaching where it is clear and consistent. Software-transcription errors and statements that conflict with the final slide content are not tested as core facts. Advanced Distinction questions use external authoritative sources and are scored separately.

Items intentionally not tested as exact core facts because the source material contains internal variation: exact Prohilo tissue-plane wording; the 1 cm versus 1.5 cm verbal tragus distance; exact per-papule K-Lumi volume; whether every eye/lip/face booster combination should occur on the same day; and whether every 2.5 mL lip syringe must always be deposited entirely within the lips. These should be standardized in the clinical SOP before being added to future versions.

Section A - MCQ answer key and teaching rationales

Q	Ans	Q	Ans	Q	Ans	Q	Ans
1	B	11	B	21	B	31	B
2	C	12	B	22	B	32	B
3	C	13	A	23	B	33	B
4	C	14	C	24	C	34	B
5	C	15	C	25	B	35	C
6	C	16	C	26	C	36	C
7	B	17	B	27	C	37	B
8	C	18	A	28	B	38	B
9	B	19	B	29	A	39	B
10	B	20	B	30	C	40	B

1. A client arrives asking specifically for the "strongest imported booster" because it is trending online. According to the Venus K-Glow prescribing framework, what should the doctor do first?

- A. Select the most premium product so the client feels fully treated.
- B. Identify the dominant skin concern, select the correct K-Glow category, and then choose an approved product.
- C. Offer whichever approved product is currently in stock.
- D. Allow the client to choose the brand after reviewing the price sheet.

Correct answer: B

Rationale: Venus K-Glow is a treatment ecosystem. The clinical concern determines the treatment category; the doctor then selects an approved product according to suitability, area and current Venus availability.

Why A is incorrect: Premium positioning does not replace diagnosis.

Why C is incorrect: Stock availability must not drive prescribing.

Why D is incorrect: The doctor, not the client, is responsible for treatment selection.

Domain: Indications & patient selection **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 8-9; transcript pp. 1-2.

2. A 49-year-old patient has prominent lower-face jowling and asks for a K-Glow booster to lift the jawline. Her skin is mildly dry, but advanced laxity is the dominant concern. What is the best first-line direction?

- A. K-Colla Bounce alone because collagen treats all laxity.
- B. K-Glass Hydration because dehydration is also present.
- C. Energy-based tightening or surgical assessment, with K-Glow considered only for an additional skin-quality objective.
- D. K-PDRN Reset because it produces structural lifting.

Correct answer: C

Rationale: Advanced laxity or jowling is not primarily corrected by a skin booster. The dominant concern should be directed to energy-based tightening or surgical assessment; a booster may be added only for a separate skin-quality need.

Why A is incorrect: K-Colla is for early dermal-quality concerns, not advanced structural laxity.

Why B is incorrect: Mild dehydration is secondary and should not distract from the dominant problem.

Why D is incorrect: K-PDRN supports repair and vitality; it is not structural lifting.

Domain: Indications & patient selection **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slide 12.

3. A patient requests K-PDRN Reset over an area with active inflammatory acne, crusting and tenderness. What is the most appropriate decision?

- A. Proceed with shallow microneedling because PDRN reduces inflammation.
- B. Proceed only if the patient signs an additional consent.
- C. Delay the booster and manage the active inflammatory or infectious condition first.
- D. Change to K-Exo Glow on the same day because exosomes support recovery.

Correct answer: C

Rationale: Active infection, inflammatory acne, dermatitis, open wounds or severe irritation are reasons to avoid or delay treatment because needling may worsen inflammation or spread infection.

Why A is incorrect: A proposed benefit does not override an active contraindication.

Why B is incorrect: Consent does not make an unsuitable treatment appropriate.

Why D is incorrect: Changing the booster does not remove the procedural risk.

Domain: Contraindications & precautions **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 11-13.

4. During consultation, a breastfeeding patient asks for a facial K-Glow booster. Which response best reflects the Venus policy taught in the training?

- A. Proceed with any non-injectable booster because it is topical.
- B. Proceed only with PDRN because it is DNA-derived.
- C. Do not prescribe the treatment during pregnancy or breastfeeding under the current Venus policy.
- D. Proceed if the patient has no previous allergy.

Correct answer: C

Rationale: Although the slide notes that product IFU and senior approval may affect general decision-making, the verbal Venus policy taught in this session was not to prescribe K-Glow boosters during pregnancy or breastfeeding.

Why A is incorrect: Microneedling-assisted delivery is still a medical procedure and product exposure remains relevant.

Why B is incorrect: PDRN is not automatically suitable in breastfeeding.

Why D is incorrect: Absence of allergy does not remove this policy restriction.

Domain: Contraindications & precautions **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slide 13; transcript p. 2.

5. A product brochure describes a preparation as injectable, but the current Venus protocol lists it only for microneedling. What should the doctor do?

- A. Inject it because the manufacturer route is more important than the clinic protocol.
- B. Use whichever route produces faster results.
- C. Do not improvise; use a route that is supported by the product IFU and the current Venus-approved protocol.
- D. Dilute it with saline and inject superficially.

Correct answer: C

Rationale: The route must match both product documentation and the current Venus protocol. Doctors should not invent a route, dilution or technique because a product contains a familiar ingredient.

Why A is incorrect: Venus approval and training requirements still apply.

Why B is incorrect: Speed of result is not a route-selection criterion.

Why D is incorrect: Unapproved dilution and injection are specifically contrary to protocol.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 14, 37, 39, 47 and 58.

6. During a microneedling-assisted booster, the skin has developed uniform mild-to-moderate erythema without significant bleeding. The doctor considers increasing depth until there is aggressive bleeding. What is the correct action?

- A. Increase depth because bleeding confirms product penetration.
- B. Continue only until pinpoint bleeding is present across the entire face.
- C. Recognize that the intended endpoint has been reached and avoid unnecessary over-treatment.
- D. Stop and repeat the entire procedure one week later.

Correct answer: C

Rationale: For K-Glow microneedling protocols, the intended endpoint is controlled erythema, not aggressive bleeding. More bleeding does not equal better collagen induction or better booster delivery.

Why A is incorrect: Aggressive bleeding increases trauma and complication risk.

Why B is incorrect: Pinpoint bleeding is not required across all areas for these booster protocols.

Why D is incorrect: There is no reason to repeat an adequately completed procedure after one week.

Domain: Treatment protocol & technique **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 14, 39, 40 and 47.

7. Which record best meets the minimum Venus documentation standard after a K-Glow treatment?

- A. Treatment category and doctor signature only.
- B. Product name, batch/expiry, quantity, route or device settings, treatment area, reaction/notes and doctor details.
- C. A photograph and the payment receipt only.
- D. Product brand and the patient's verbal confirmation that the treatment felt comfortable.

Correct answer: B

Rationale: Traceability requires the actual product, batch and expiry details, quantity, technique or device settings, treatment area, reaction/doctor notes and responsible doctor. Consent, photographs and written post-care are also part of the broader standard.

Why A is incorrect: This does not provide product traceability or procedural detail.

Why C is incorrect: Payment documentation is not a clinical treatment record.

Why D is incorrect: Comfort alone does not document what was administered.

Domain: Post-care, consent & documentation **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 17, 90 and 96; transcript pp. 20-21.

8. Which post-treatment finding should be treated as an urgent red flag rather than an expected temporary reaction?

- A. Small injection papules that are gradually settling.
- B. Mild lip sensitivity on the day of treatment.
- C. Increasing severe pain with blanching and delayed capillary refill.
- D. Temporary under-eye puffiness without pain or visual change.

Correct answer: C

Rationale: Severe or increasing pain, blanching, dusky discoloration, livedo and delayed capillary refill may indicate vascular compromise and require immediate action.

Why A is incorrect: Temporary papules are expected after some injectables.

Why B is incorrect: Mild transient sensitivity is common after lip treatment.

Why D is incorrect: Temporary puffiness can be expected, although worsening swelling or eye symptoms require review.

Domain: Complications & emergency management **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slides 18 and 31.

9. Which patient is the best fit for K-Glass Hydration as the primary treatment?

- A. A patient seeking major cheek projection and correction of structural volume loss.
- B. A patient with dull, dehydrated, mildly crepey skin and fine dehydration lines.
- C. A patient with severe inflammatory acne and open lesions.
- D. A patient whose main concern is advanced jowling.

Correct answer: B

Rationale: K-Glass is intended for hydration, luminosity, fine dehydration lines and early skin-quality concerns. It is not a volumizing filler or a substitute for structural correction.

Why A is incorrect: Structural volume loss requires a filler or other structural strategy.

Why C is incorrect: Active inflammation is a reason to delay.

Why D is incorrect: Advanced laxity requires another first-line direction.

Domain: Indications & patient selection **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slide 20.

10. Which statement most accurately distinguishes a conventional hydration skin booster from bioremodelling as taught in the presentation?

- A. Every non-cross-linked HA booster is automatically a bioremodelling product.
- B. A skin booster mainly targets hydration and texture, whereas bioremodelling also aims to improve broader tissue firmness, elasticity and structure without acting as a traditional filler.
- C. Bioremodelling is simply a lower-dose dermal filler technique.
- D. Skin boosters add volume, whereas bioremodelling only brightens the skin.

Correct answer: B

Rationale: The training distinguishes hydration-focused skin boosters from products such as Profhilo that combine hydration with a broader tissue-quality and elasticity objective. Bioremodelling is not conventional volumizing filler.

Why A is incorrect: Not every skin booster has bioremodelling properties.

Why C is incorrect: Bioremodelling is not defined as low-dose filler.

Why D is incorrect: The direction of these effects is inaccurate.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Concept

Source: Training Presentation slide 24.

11. Which description matches Profhilo in the K-Glass product table?

- A. 40 mg cross-linked HA in 1 mL designed for focal volume replacement.
- B. 64 mg non-cross-linked HA in 2 mL, combining high- and low-molecular-weight HA in a thermally stabilized hybrid complex.
- C. 68 mg cross-linked HA with succinic acid in a 2 mL gel filler.
- D. 20 mg PN in 2 mL for periocular injection.

Correct answer: B

Rationale: Profhilo is presented as 64 mg non-cross-linked HA in a 2 mL syringe, with high- and low-molecular-weight HA in a thermally stabilized hybrid complex.

Why A is incorrect: This does not match the product composition or objective.

Why C is incorrect: Lamist contains HA plus succinic acid and is not described as a cross-linked filler.

Why D is incorrect: This describes an under-eye PN-type quantity, not Profhilo.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Recall + interpretation

Source: Training Presentation slide 22.

12. Under the standard facial BAP protocol, how is a complete 2 mL syringe distributed?

- A. Five points total at 0.4 mL per point.
- B. Ten points total at 0.2 mL per point.
- C. Ten points total at 0.1 mL per point, leaving 1 mL for maintenance.
- D. Twenty points total at 0.1 mL per point.

Correct answer: B

Rationale: Five BAP points are used on each side of the face, giving ten points total. At 0.2 mL per point, the complete 2 mL syringe is used.

Why A is incorrect: The standard training uses five points per side, not five total.

Why C is incorrect: This is the half-syringe practice specifically rejected by Venus.

Why D is incorrect: The standard facial BAP plan is ten points, not twenty.

Domain: Treatment protocol & technique **Difficulty:** Moderate **Type:** Calculation

Source: Training Presentation slide 25.

13. A doctor marks the tragus BAP point directly at the inferior tragus. According to the presentation marking guide, what correction is required?

- A. Move the point approximately 1 cm anterior to the inferior tragus.
- B. Move the point 2 cm posterior to the tragus.
- C. Move the point to the midpoint between the tragus and lateral canthus.
- D. Keep the point at the inferior tragus because that is the standard landmark.

Correct answer: A

Rationale: The presentation specifies that the tragus point should be at least 1 cm anterior to the inferior tragus.

Why B is incorrect: This would move the point posteriorly rather than into the taught treatment area.

Why C is incorrect: This is not the stated marking rule.

Why D is incorrect: The point is not marked directly on the tragus.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied anatomy

Source: Training Presentation slide 26.

14. A patient asks to buy only half a Profilo facial treatment and return later for the remaining product. What is the correct Venus response?

- A. Use 1 mL now and refrigerate the remainder for the same patient.
- B. Use 1 mL now and share the remainder with another patient the same day.
- C. Explain that the standard facial protocol uses the complete 2 mL syringe for one patient, with no half syringe, sharing or leftover use.
- D. Dilute the 1 mL with saline so all BAP points can be treated.

Correct answer: C

Rationale: The training expressly requires a complete 2 mL syringe for one patient and rejects half-syringe, sharing and leftover use.

Why A is incorrect: Opened product is not retained under the taught policy.

Why B is incorrect: Sharing creates sterility and traceability concerns.

Why D is incorrect: Unapproved dilution changes the product and protocol.

Domain: Treatment protocol & technique **Difficulty:** Moderate **Type:** Applied policy

Source: Training Presentation slides 25 and 33.

15. Immediately after an HA booster injection, the patient develops disproportionate pain, blanching and a mottled pattern. What is the best immediate response?

- A. Apply a cold compress and allow the patient to leave with reassurance.
- B. Continue injecting slowly to complete the syringe and reassess afterward.
- C. Stop injecting, keep the patient in the clinic, assess colour/pain/capillary refill/vision, call the senior doctor and initiate the Venus HA vascular protocol.
- D. Massage only and review the patient after 24 hours.

Correct answer: C

Rationale: The training requires immediate cessation, on-site assessment, escalation and prompt initiation of the HA vascular-compromise protocol. The patient should not be discharged while the concern is unresolved.

Why A is incorrect: Cold compression and discharge are inappropriate for suspected vascular compromise.

Why B is incorrect: Continuing injection may worsen the event.

Why D is incorrect: Massage alone and delayed review are insufficient.

Domain: Complications & emergency management **Difficulty:** Hard **Type:** Applied emergency

Source: Training Presentation slide 31; transcript pp. 8-9.

16. A patient reports blurred vision and severe headache immediately after an injectable HA treatment. Which action has the highest priority?

- A. Observe for 30 minutes because visual symptoms are often anxiety-related.
- B. Give oral analgesia and arrange a routine next-day review.
- C. Treat this as an emergency and arrange urgent ophthalmology/hospital referral while activating the clinic escalation pathway.
- D. Inject additional HA to test whether the symptoms worsen.

Correct answer: C

Rationale: Any visual symptom after an injectable is a high-urgency emergency. The presentation calls for urgent ophthalmology/hospital referral and immediate escalation.

Why A is incorrect: Observation alone risks irreversible delay.

Why B is incorrect: Routine review is inadequate.

Why D is incorrect: Further injection is unsafe.

Domain: Complications & emergency management **Difficulty:** Hard **Type:** Applied emergency

Source: Training Presentation slide 31.

17. Which statement best reflects the final Venus injectable technique message?

- A. A negative aspiration result proves that an injection is safe.
- B. Aspiration may be used according to clinic protocol, but it does not replace anatomy, correct plane, small controlled dosing, slow low-pressure injection and continuous observation.
- C. Aspiration should never be considered under any circumstance.
- D. Needle length alone determines whether vascular compromise can occur.

Correct answer: B

Rationale: The presentation includes aspiration among practical steps but the final safety note states that aspiration is not a reliable universal safety test. It must not replace anatomy, technique and observation.

Why A is incorrect: A negative aspiration does not guarantee safety.

Why C is incorrect: The training does not prohibit aspiration; it places it in context.

Why D is incorrect: Vascular risk is not determined by needle length alone.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied safety

Source: Training Presentation slides 33 and 96.

18. Which presentation best fits K-Colla Bounce?

- A. Early laxity, crepey texture, fine lines and reduced skin resilience without major structural volume loss.
- B. A deep static fold that requires immediate projection.
- C. An active infected acne cyst.
- D. A large tear-trough hollow requiring structural assessment.

Correct answer: A

Rationale: K-Colla focuses on gradual improvement in firmness, elasticity, texture and skin bounce. It is not a filler, surgery or scar-specific replacement.

Why B is incorrect: Immediate projection is not the aim.

Why C is incorrect: Active infection is a contraindication.

Why D is incorrect: Structural hollowing requires a different assessment.

Domain: Indications & patient selection **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slide 35.

19. Which statement about collagen types matches the training?

- A. Type I mainly provides finer early repair support, while Type III provides most tensile strength.
- B. Type I supports firmness and strength, while Type III provides finer structural support and contributes to early repair.
- C. Type I and Type III are interchangeable marketing terms with no structural distinction.
- D. Recombinant collagen is harvested directly from the patient immediately before treatment.

Correct answer: B

Rationale: The presentation states that Type I supports skin firmness and strength, while Type III provides finer structural support and has an important role in early repair. Recombinant collagen is bioengineered through fermentation.

Why A is incorrect: The type functions are reversed.

Why C is incorrect: The presentation expressly distinguishes their roles.

Why D is incorrect: This is not how recombinant collagen is described.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Concept

Source: Training Presentation slide 36.

20. What is the Venus-approved protocol for SeduSkin Hylagen3 in the current K-Colla category?

- A. Inject the 5 mL vial using BAP points.
- B. Use the complete 5 mL vial with microneedling; do not inject it.
- C. Use half the vial and store the remainder.
- D. Apply it only after lip microneedling.

Correct answer: B

Rationale: SeduSkin Hylagen3 is listed as a 5 mL product used per client through the Venus-approved microneedling protocol and is specifically marked "Do not inject."

Why A is incorrect: The current Venus protocol does not allow injection.

Why C is incorrect: The training specifies the complete vial per client.

Why D is incorrect: It is a facial collagen-category product, not a lip-only product.

Domain: Treatment protocol & technique **Difficulty:** Moderate **Type:** Product-specific

Source: Training Presentation slide 37.

21. A doctor argues that every collagen product should be microneedled because K-Colla is a microneedling category. Which response is most accurate?

- A. Correct; ingredients determine that every collagen product can be microneedled.
- B. Incorrect; route is product-specific. Rejucoll is currently microneedled at Venus, while Karisma has its own injection technique and should not be treated as the same product.
- C. Incorrect; all collagen products should instead be injected through the Profhilo BAP points.
- D. Correct, provided the depth is at least 2.5 mm.

Correct answer: B

Rationale: The training repeatedly states not to use one technique for every collagen product. Current Venus use and the product IFU must determine the route.

Why A is incorrect: A collagen ingredient does not automatically define route.

Why C is incorrect: The BAP technique is not a universal collagen protocol.

Why D is incorrect: Depth does not make an unapproved route acceptable.

Domain: Contraindications & precautions **Difficulty:** Hard **Type:** Applied product route

Source: Training Presentation slides 37 and 39.

22. A patient with a darker, PIH-prone skin type is receiving a microneedling-assisted booster. Which adjustment best follows the training?

- A. Increase depth and passes because darker skin requires more stimulation.
- B. Use conservative settings, avoid unnecessary trauma and stop at the intended erythema endpoint.
- C. Aim for aggressive bleeding to reduce the risk of pigmentation.
- D. Apply a strong acid immediately afterward to prevent PIH.

Correct answer: B

Rationale: The training advises conservative depth and passes in darker or PIH-prone skin and warns that over-treatment can increase inflammatory and pigmentary risk.

Why A is incorrect: More trauma may increase PIH risk.

Why C is incorrect: Aggressive bleeding is not the target endpoint.

Why D is incorrect: Strong irritants should not be casually combined with needling.

Domain: Complications & emergency management **Difficulty:** Hard **Type:** Applied precaution

Source: Training Presentation slides 39-40.

23. Which comparison between PN and PDRN is consistent with the training?

- A. PN consists of shorter fragments and is always more fluid than PDRN.
- B. PN generally has longer chains and greater viscoelastic support, while PDRN consists of shorter fragments with a repair/recovery emphasis.
- C. PDRN is a cross-linked HA gel and PN is a collagen peptide.
- D. PN and PDRN are identical terms and should be used interchangeably.

Correct answer: B

Rationale: The training distinguishes PN as longer-chain, more cushioning/viscoelastic material and PDRN as shorter DNA fragments with a stronger repair and recovery emphasis.

Why A is incorrect: The chain-length and tissue-behaviour description is reversed.

Why C is incorrect: Neither term describes HA gel or collagen peptide.

Why D is incorrect: The presentation specifically differentiates them.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Concept

Source: Training Presentation slide 43.

24. Which explanation of PDRN is most appropriate for a patient?

- A. It is fish oil that permanently replaces damaged DNA.
- B. It is a stem-cell treatment that becomes new skin cells.
- C. It is a purified DNA-fragment ingredient used to support repair and recovery; it is not filler, fish oil, stem cells or guaranteed permanent regeneration.
- D. It is a muscle-relaxing toxin used for pore control.

Correct answer: C

Rationale: The training emphasizes accurate, restrained communication: PDRN is described as a DNA-fragment-based repair-supporting booster, not fish oil, filler, stem cells or permanent regeneration.

Why A is incorrect: This is scientifically and clinically misleading.

Why B is incorrect: PDRN is not stem-cell therapy.

Why D is incorrect: This describes a different treatment concept.

Domain: Scientific foundation & mechanism **Difficulty:** Moderate **Type:** Communication

Source: Training Presentation slide 44.

25. Which plan follows the current Venus K-PDRN protocol?

- A. Inject Lapuroon facially because injectable delivery is always more effective.
- B. Microneedle the approved PDRN product using a controlled endpoint, or apply topically after RF microneedling where protocol allows; do not mix it with PRP, saline or another booster unless specifically approved.
- C. Mix PDRN with PRP to increase growth-factor concentration.
- D. Dilute PDRN with saline so it covers a larger area.

Correct answer: B

Rationale: The current Venus facial K-PDRN route is microneedling-assisted, with possible post-RF topical application where supported. Improvised mixing or dilution is not permitted.

Why A is incorrect: The current Venus facial protocol is not routine injection.

Why C is incorrect: The training specifically says not to mix without an approved protocol.

Why D is incorrect: Unapproved dilution changes the protocol.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slide 47; transcript pp. 11-12.

26. A patient reports a previous generalized allergic reaction after an unknown "salmon DNA" injection. She now requests K-PDRN Reset. What is the safest next step?

- A. Proceed because a previous reaction proves the product was active.
- B. Proceed with half the dose after oral antihistamine.
- C. Do not proceed routinely; clarify the prior reaction, ask specifically about fish/salmon/seafood allergy and obtain senior assessment before any decision.
- D. Change the product brand but keep the same treatment category without further assessment.

Correct answer: C

Rationale: The training requires specific allergy and previous-reaction screening before PN/PDRN. A prior severe reaction warrants caution and senior review rather than dose reduction or brand substitution.

Why A is incorrect: A reaction is not evidence of efficacy.

Why B is incorrect: Pre-medication does not make an unclear severe reaction safe.

Why D is incorrect: A different brand may still contain a related source material.

Domain: Contraindications & precautions **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slides 13 and 47.

27. A patient purchased Rejuran Healer, but an assistant brings a syringe of Lapuroon Super into the room. What should the doctor do?

- A. Use it because both fall under K-PDRN Reset.
- B. Use it if the volume is similar and document the change afterward.
- C. Stop and verify the prescribed and purchased product, variant, quantity, batch and expiry before treatment; do not substitute without an authorized clinical and commercial decision.
- D. Mix the two products so the patient receives benefits of both.

Correct answer: C

Rationale: The doctor must verify that the actual product matches the prescribed and sold treatment, then document the exact product, quantity, batch and route. Category similarity does not permit silent substitution.

Why A is incorrect: Products within one category are not interchangeable without review.

Why B is incorrect: Similarity of volume is not a substitution rule.

Why D is incorrect: Unapproved mixing is prohibited.

Domain: Post-care, consent & documentation **Difficulty:** Hard **Type:** Product verification

Source: Training Presentation slides 17, 45 and 90; transcript pp. 14-15.

28. Which description of exosomes is most accurate within the training framework?

- A. Living stem cells that permanently engraft in the skin.
- B. Cell-free extracellular vesicles that carry signalling cargo such as proteins, lipids, growth-related molecules and RNA.
- C. Cross-linked HA particles used for volume replacement.
- D. Botulinum toxin fragments that relax facial muscles.

Correct answer: B

Rationale: Exosomes are presented as cell-free extracellular vesicles involved in cell-to-cell signalling. They do not become new cells and should not be called stem-cell therapy.

Why A is incorrect: Exosomes are not living stem cells.

Why C is incorrect: They are not HA fillers.

Why D is incorrect: They are not toxin products.

Domain: Scientific foundation & mechanism **Difficulty:** Moderate **Type:** Concept

Source: Training Presentation slides 52 and 59.

29. Which product-source and preparation pairing matches the current K-Exo training?

- A. Exoxe - human amniotic-fluid stem-cell source; reconstitute the lyophilized vial with the supplied diluent for the skin protocol.
- B. Exoxe - salmon DNA source; inject without reconstitution.
- C. ASCE+ - bovine collagen source; dilute with PRP.
- D. ASCE+ - botulinum peptide source; use as intramuscular toxin.

Correct answer: A

Rationale: The presentation lists Exoxe as derived from human amniotic-fluid stem-cell preparations and ASCE+ as derived from human adipose tissue. Both skin preparations are reconstituted with the supplied diluent according to protocol.

Why B is incorrect: Exoxe is not a PDRN product and requires preparation.

Why C is incorrect: ASCE+ is not described as bovine collagen and should not be mixed with PRP.

Why D is incorrect: It is not a toxin product.

Domain: Post-care, consent & documentation **Difficulty:** Hard **Type:** Product-specific

Source: Training Presentation slides 55-57.

30. Which statement should a Venus doctor avoid when counselling a K-Exo Glow patient?

- A. The aim is recovery support and gradual skin-quality improvement.
- B. Product source, manufacturing, storage and intended route affect quality.
- C. This is stem-cell therapy that repairs DNA and permanently reverses ageing.
- D. The evidence is promising, but products and outcomes vary.

Correct answer: C

Rationale: The training prohibits stem-cell, DNA-repair, cure and permanent anti-ageing claims. Exosomes support signalling; they do not become new cells.

Why A is incorrect: This is a restrained description consistent with the training.

Why B is incorrect: This is an important quality-control point.

Why D is incorrect: The presentation explicitly acknowledges variable evidence and quality.

Domain: Contraindications & precautions **Difficulty:** Hard **Type:** Communication

Source: Training Presentation slides 54, 58 and 59.

31. A patient requests K-Lumi Eyes for dark circles. Examination shows marked tear-trough hollowing, fat prolapse and chronic malar edema after previous filler. What is the best response?

- A. Proceed with large PN boluses because the product will replace the lost volume.
- B. Explain that K-Lumi is not the correct primary solution, assess the structural and edema causes, and delay or redirect treatment.
- C. Microneedle aggressively because microneedling avoids all under-eye risk.
- D. Inject hyaluronidase into the PN product before treatment.

Correct answer: B

Rationale: K-Lumi is for selected under-eye skin-quality concerns. Severe bags, structural hollowing, unresolved swelling and prior filler-related edema require careful assessment and often another treatment direction.

Why A is incorrect: PN boosters are not structural fillers.

Why C is incorrect: Microneedling does not make an unsuitable diagnosis appropriate.

Why D is incorrect: Hyaluronidase is not mixed into PN products.

Domain: Indications & patient selection **Difficulty:** Hard **Type:** Applied diagnosis

Source: Training Presentation slides 61 and 64.

32. Which under-eye injection approach best reflects the K-Lumi safety slide?

- A. Deep orbital placement at 90 degrees with large boluses.
- B. Superficial placement in the papillary dermis using a conservative product-specific technique, approximately 10-20 degrees, while avoiding large boluses and the lash line.
- C. Subperiosteal placement using the entire needle length.
- D. Intramuscular injection to reduce orbicularis activity.

Correct answer: B

Rationale: The training specifies a superficial, conservative periocular technique in the papillary dermis at a low angle, avoiding deep orbital injection, large boluses and unsupported lash-line treatment.

Why A is incorrect: This is the opposite of the taught approach.

Why C is incorrect: Subperiosteal placement is not the booster protocol.

Why D is incorrect: K-Lumi is not a neuromodulator treatment.

Domain: Treatment protocol & technique **Difficulty:** Hard **Type:** Applied technique

Source: Training Presentation slide 65.

33. After a pure PN under-eye booster, the patient develops severe swelling and eye pain. Which statement is correct?

- A. Hyaluronidase will dissolve the PN product, so no further assessment is required.
- B. Eye pain or severe swelling requires urgent medical review, and hyaluronidase does not dissolve pure PN/PDRN products.
- C. This is always normal for 7-10 days.
- D. Massage the area forcefully until the swelling disappears.

Correct answer: B

Rationale: The presentation identifies eye pain, visual symptoms and severe swelling as urgent concerns. It also states that hyaluronidase does not dissolve pure PN/PDRN.

Why A is incorrect: The enzyme targets HA, not pure PN/PDRN.

Why C is incorrect: Severe or worsening symptoms should not be normalized.

Why D is incorrect: Forceful manipulation is not the emergency plan.

Domain: Complications & emergency management **Difficulty:** Hard **Type:** Applied emergency

Source: Training Presentation slide 65.

34. Which expectation is appropriate for K-Lip Dew?

- A. Major lip enlargement and Russian-lip reshaping.
- B. Hydration, softness, improvement in fine lip lines and a natural sheen without a filler look.
- C. Correction of severe filler migration.
- D. Permanent pink pigmentation of the lips.

Correct answer: B

Rationale: K-Lip Dew is a lip-quality treatment, not a volumizing filler, reshaping procedure or permanent pigmentation treatment.

Why A is incorrect: This is a filler-type expectation.

Why C is incorrect: Problematic filler should be assessed and corrected separately.

Why D is incorrect: The training specifically warns against promising pink colour.

Domain: Indications & patient selection **Difficulty:** Moderate **Type:** Applied

Source: Training Presentation slide 70; transcript pp. 16-18.

35. A patient had lip filler 5 days ago and still has bruising, swelling and a palpable nodule. She asks for K-Lip Dew today. What is the best decision?

- A. Proceed because a booster will smooth the nodule.
- B. Proceed with half the usual volume.
- C. Delay the booster, assess the existing filler and wait until the lips have settled; a practical interval is often about 2-4 weeks when no complication is present.
- D. Dissolve all filler and perform the booster in the same sitting without further assessment.

Correct answer: C

Rationale: Recent filler with ongoing swelling, bruising, nodules, migration or unexplained pain is a reason to delay. The training suggests reassessment after the filler has settled, commonly around 2-4 weeks.

Why A is incorrect: A booster is not a treatment for a filler nodule.

Why B is incorrect: Dose reduction does not remove the underlying concern.

Why D is incorrect: Correction decisions must be individualized and staged appropriately.

Domain: Contraindications & precautions **Difficulty:** Hard **Type:** Applied

Source: Training Presentation slide 76; transcript p. 18.

36. Which lip-booster practice is compliant with the training?

- A. Reuse the same stamp tip on the same patient at the next monthly session.
- B. Share an opened syringe between two patients if a new tip is used.
- C. Use a new sterile product-specific needle or tip for every treatment, document quantity, and do not share an opened syringe unless the IFU explicitly permits it.
- D. Store all opened lip boosters for later because they contain HA.

Correct answer: C

Rationale: The training requires a new sterile tip for each treatment and prohibits unsafe syringe sharing or storage practices that are not explicitly supported by the IFU.

Why A is incorrect: A new session requires a new sterile tip.

Why B is incorrect: Changing the tip does not make multi-patient sharing acceptable.

Why D is incorrect: Opened-product storage is not assumed safe.

Domain: Complications & emergency management **Difficulty:** Moderate **Type:** Applied safety

Source: Training Presentation slide 75.

37. How does the objective of K-Tox Refine differ from conventional injectable Botulinum Toxin Type A treatment?

- A. K-Tox is intended mainly for clinically significant muscle paralysis.
- B. K-Tox focuses on pore appearance, sebum and superficial texture or fine lines rather than full muscle relaxation.
- C. K-Tox is a structural filler for volume loss.
- D. There is no difference; the terms are interchangeable.

Correct answer: B

Rationale: K-Tox Refine is positioned as a superficial skin-refinement protocol. The current Lumi Meso Tox product contains botulinum-derived peptides and is not a substitute for injectable Botulinum Toxin Type A.

Why A is incorrect: The training states that clinically significant muscle relaxation is not the objective.

Why C is incorrect: K-Tox is not a filler.

Why D is incorrect: The distinction is central to the module.

Domain: Scientific foundation & mechanism **Difficulty:** Hard **Type:** Concept

Source: Training Presentation slides 78-82.

38. What is the current Venus approach to Lumi Meso Tox under K-Tox Refine?

- A. Inject the product intramuscularly in standard Botox points.
- B. Use the complete 5 mL vial with the approved microneedling protocol; it may also be applied after RF microneedling where the clinic protocol permits.
- C. Use 0.5 mL and store the remainder for another patient.
- D. Mix it with injectable Botox to guarantee pore closure.

Correct answer: B

Rationale: The presentation lists Lumi Meso Tox as a 5 mL microneedling product, with possible post-RF microneedling use. It is not an intramuscular Botox substitute and should not be casually mixed.

Why A is incorrect: This is not the taught route or objective.

Why C is incorrect: The training specifies the complete vial per client.

Why D is incorrect: Unapproved mixing is not permitted.

Domain: Treatment protocol & technique **Difficulty:** Moderate **Type:** Product-specific

Source: Training Presentation slides 80-83.

39. Which treatment-course entry should be written on the doctor sheet?

- A. Profhilo: one session only; all other boosters: one session only.
- B. Profhilo: two sessions at one-month intervals plus ongoing maintenance around every six months; other K-Glow boosters: three to four monthly sessions plus maintenance.
- C. All boosters: weekly sessions until visible peeling occurs.
- D. Profhilo: four sessions in one month; other boosters: no maintenance.

Correct answer: B

Rationale: The presentation standardizes Profhilo as two sessions one month apart, with maintenance, while other boosters generally follow a three-to-four-session monthly course with ongoing maintenance.

Why A is incorrect: One session is not the taught course.

Why C is incorrect: Weekly treatment and peeling are not the protocol.

Why D is incorrect: This reverses the taught schedule.

Domain: Post-care, consent & documentation **Difficulty:** Hard **Type:** Applied course planning

Source: Training Presentation slide 88; transcript p. 20.

40. Which aftercare plan is most consistent with the universal K-Glow post-treatment guidance?

- A.** Massage all treated areas frequently, resume retinol the same evening and use a hot shower to reduce bumps.
- B.** Avoid touching, rubbing, makeup and excessive heat initially; use gentle skincare and sun protection; avoid strong actives for several days and other irritating procedures for about 1-2 weeks unless advised.
- C.** Apply bleaching cream immediately to prevent pigmentation.
- D.** Schedule an aggressive peel within 48 hours to improve penetration.

Correct answer: B

Rationale: The universal post-care emphasizes protection of the treated barrier: no rubbing or manipulation, temporary avoidance of makeup, heat and strenuous activity, gentle skincare, delayed strong actives and avoidance of irritating procedures while healing.

Why A is incorrect: Massage, retinoids and heat may increase irritation.

Why C is incorrect: Bleaching creams are specifically avoided during early recovery.

Why D is incorrect: An early aggressive peel may worsen inflammation.

Domain: Post-care, consent & documentation **Difficulty:** Hard **Type:** Applied post-care

Source: Training Presentation slides 91 and 96; transcript p. 21.

Section B - Written-answer model responses and marking rubrics

Award 1 mark for each essential point. Accept clinically equivalent wording when the meaning is clear. Do not award marks for vague statements that omit the required decision or action. Maximum 4 marks per question.

41. Protocol reconstruction - K-Glass facial BAP treatment

A suitable patient has selected a full-face Profhilo K-Glass treatment. Write the essential sequence from preparation to completion. Your answer should include four clinically important steps or controls.

Mark	Essential point
1	Confirm indication/contraindications, informed consent, standardized photographs and product verification (authenticity, integrity, batch and expiry).
1	Apply selective anaesthesia as required, remove it completely, disinfect, and mark five BAP points on each side before opening/using the syringe.
1	Use the complete 2 mL syringe for one patient: 0.2 mL at each of ten points; prime the needle, use the correct plane/angle, inject slowly with controlled low pressure and observe the skin.
1	Document product, batch, quantity and technique; provide written post-care, explain expected papules and red flags, and plan the next session/review.

Source: Slides 17, 25-26, 33 and 96.

42. Patient selection - active inflammatory skin before an event

A patient has active inflammatory acne with several excoriated lesions and wants K-PDRN Reset five days before a wedding. Explain your decision and immediate plan in four points.

Mark	Essential point
1	Do not proceed over active inflammatory acne/open or excoriated lesions because needling may worsen inflammation or infection.
1	Assess for infection, dermatitis, herpes, recent procedures, medical risks, allergy history and the patient's unrealistic time-sensitive expectations.
1	Direct the patient to appropriate acne/medical dermatology management first and allow the skin to settle before reconsidering a booster.
1	Explain that K-PDRN is not an acne cure or instant event treatment; document the consultation and provide a realistic future staged plan if appropriate.

Source: Slides 11-13, 18, 42 and 47.

43. Clinical modification - under-eye diagnosis

A patient requests K-Lumi Eyes for dark circles. Examination shows major tear-trough hollowing, chronic malar edema and recent under-eye filler. State four elements of your assessment and management plan.

Mark	Essential point
1	Classify the darkness and recognize structural hollowing/edema and recent filler as dominant concerns rather than assuming a pure skin-quality problem.
1	Delay K-Lumi while swelling and recent filler status are unresolved; review filler type, date, volume and prior reactions/complications.
1	Explain that K-Lumi does not remove eye bags or replace structural correction; consider an appropriate filler-correction, edema or specialist assessment pathway.
1	If later suitable, use conservative product-specific treatment, document the plan and counsel regarding puffiness, bruising and urgent eye symptoms.

Source: Slides 61, 64-65; transcript pp. 14-16.

44. Complication management - suspected HA vascular compromise

During K-Glass injection, the patient develops severe increasing pain, blanching and delayed capillary refill. Write the first four actions that should occur.

Mark	Essential point
1	Stop injecting immediately and keep the patient in the clinic.
1	Assess and document pain, colour, capillary refill, temperature/skin changes and vision; photograph/mark the affected area where appropriate.
1	Call the senior doctor/Medical Director and activate the Venus HA vascular-compromise SOP without delay.
1	Use the emergency protocol, including prompt hyaluronidase and supportive measures as directed; arrange urgent ophthalmology/hospital referral for any visual symptom and do not discharge until appropriately stabilized or referred.

Source: Slide 31; transcript pp. 8-9.

45. Integrated treatment planning and documentation

A bride has four months before her wedding. Her dominant concerns are dehydration, visible pores and mild under-eye crepiness. She asks to complete every booster in one visit. Propose a safe, realistic plan in four points.

Mark	Essential point
1	Identify the relevant categories: K-Glass for dehydration, K-Tox Refine for pores/sebum/surface refinement, and K-Lumi Eyes for selected under-eye skin-quality concerns after diagnosis.
1	Do not stack multiple facial inflammatory procedures automatically; stage the plan according to area, skin sensitivity, downtime and clinical rationale, starting early enough before the event.
1	Explain realistic outcomes, course requirements and maintenance; boosters improve skin quality and do not guarantee complete correction.
1	Obtain consent, take standardized photographs, verify and record each product/quantity/technique at every session, and provide written aftercare and red-flag advice.

Source: Slides 61, 65, 78-79, 85-91 and 96.

Section C - Advanced Distinction answer key

46: B 47: B 48: C 49: C 50: B

46. IBSA describes NAHYCO Hybrid Technology as which type of stabilization process?

- A. Chemical BDDE cross-linking of one molecular-weight HA fraction.
- B. A patented thermal process combining high- and low-molecular-weight HA without chemical cross-linking reagents, producing cooperative hybrid complexes.
- C. Freeze-drying HA with calcium hydroxylapatite particles.
- D. Enzymatic conversion of HA into PN.

Correct answer: B

Rationale: IBSA states that its patented thermal process combines high- and low-molecular-weight HA without chemical reagents and produces cooperative hybrid complexes that are free of cross-linking chemical agents.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: IBSA Derma, "NAHYCO Hybrid Technology," accessed August 2026.

47. Which pair best summarizes the two widely described biological mechanisms of PDRN in tissue repair literature?

- A. Muscarinic receptor blockade and calcium chelation.
- B. Adenosine A2A receptor activation and provision of nucleotides/nucleosides through the salvage pathway.
- C. Chemical cross-linking and acetylcholine inhibition.
- D. Melanin oxidation and collagen denaturation.

Correct answer: B

Rationale: PDRN literature commonly describes A2A receptor engagement and provision of nucleotide building blocks through the salvage pathway as complementary mechanisms.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: Squadrito et al., "Pharmacological Activity and Clinical Use of PDRN," *Frontiers in Pharmacology* (2017); Galeano et al. review (2021).

48. According to MISEV2023 nomenclature guidance, when is the term "exosome" most scientifically defensible?

- A. Whenever a product contains particles smaller than 200 nm.
- B. Whenever a supplier uses the word on the box.
- C. When endosomal/subcellular biogenesis has been demonstrated; otherwise a broader extracellular-vesicle term is generally preferred.
- D. Only when the vesicles are derived from adipose tissue.

Correct answer: C

Rationale: MISEV2023 discourages biogenesis-based terms such as exosome unless the relevant subcellular origin can be demonstrated for the specific preparation.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: Welsh et al., MISEV2023, *Journal of Extracellular Vesicles* (2024).

49. Which statement reflects current U.S. FDA information on microneedling devices?

- A. Microneedling cartridges may be cleaned and reused on the same patient.
- B. FDA-authorized microneedling devices are approved to deliver cosmetics, drugs and PRP into the skin.
- C. A new cartridge should be used for each patient and each session, and FDA states that microneedling devices are not approved for delivery of cosmetics, topical medications, drugs or blood products into the skin.
- D. Pigment changes and infection are not recognized microneedling risks.

Correct answer: C

Rationale: The FDA advises against cartridge reuse and states that microneedling devices are not approved as delivery devices for cosmetics, topical medications, drugs or blood products. This is a U.S. regulatory statement and does not itself define Pakistan law.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: U.S. FDA, "Microneedling Devices," updated October 15, 2025.

50. Which counselling statement is most defensible when discussing human-cell- or tissue-derived "regenerative" products in light of FDA safety communications?

- A. A human tissue source proves the product is safe and approved.
- B. Unapproved products may not have been reviewed for quality, safety, purity or potency, so disease-treatment or cure claims should be avoided.
- C. The word exosome means the product is exempt from drug and biologic regulation.
- D. A high particle count alone proves clinical efficacy.

Correct answer: B

Rationale: FDA warns that unapproved products made from human cells or tissues may not have been reviewed or verified for quality, safety, purity or potency. Product count or source alone does not prove approval or efficacy.

Domain: Advanced Distinction **Difficulty:** Advanced **Type:** Advanced

Source: U.S. FDA patient and consumer warning on unapproved products from human cells or tissues, May 11, 2026; FDA exosome safety communications.

Question blueprint and domain map

Domain	Questions	Count
Scientific foundation & mechanism	10, 11, 19, 23, 24, 28, 37	7
Indications & patient selection	1, 2, 9, 18, 31, 34	6
Contraindications & precautions	3, 4, 21, 26, 30, 35	6
Treatment protocol & technique	5, 6, 12, 13, 14, 17, 20, 25, 32, 38	10
Complications & emergency management	8, 15, 16, 22, 33, 36	6
Post-care, consent & documentation	7, 27, 29, 39, 40	5

Application emphasis: 27 of 40 MCQs are explicitly case- or application-based. Additional questions require interpretation rather than simple recall.

External references used only for Advanced Distinction questions

1. IBSA Derma. NAHYCO Hybrid Technology. Official manufacturer educational page; accessed August 2026.
2. Squadrito F, et al. Pharmacological Activity and Clinical Use of PDRN. *Frontiers in Pharmacology*. 2017.
3. Galeano M, et al. Polydeoxyribonucleotide: A Promising Biological Platform to Accelerate Impaired Skin Wound Healing. 2021 review.
4. Welsh JA, et al. Minimal Information for Studies of Extracellular Vesicles (MISEV2023). *Journal of Extracellular Vesicles*. 2024.
5. U.S. Food and Drug Administration. Microneedling Devices. Updated October 15, 2025.
6. U.S. Food and Drug Administration. Patient and Consumer Warning about Potential Serious Risks of Harm following Use of Unapproved Products from Human Cells or Tissues. May 11, 2026.

Venus K-Glow Boosters

Result, Domain Scorecard and Retest Record | Version A

Assessment code	VA-KGLOW-A-2026.08
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Candidate and attempt details

Doctor full name		Employee ID	
Branch / city		Assessment date	
Assessment version	A	Examiner	
Attempt number		Retest version if applicable	

Score calculation

Section	Maximum	Score obtained
Core and applied MCQs	80	
Written clinical questions	20	
Core assessment total	100	
Advanced Distinction	5	

Final core percentage: _____ % Advanced score: _____ / 5

MCQ domain performance

Domain	Question numbers	Correct	Total	%
Scientific foundation & mechanism	10, 11, 19, 23, 24, 28, 37		7	
Indications & patient selection	1, 2, 9, 18, 31, 34		6	
Contraindications & precautions	3, 4, 21, 26, 30, 35		6	
Treatment protocol & technique	5, 6, 12, 13, 14, 17, 20, 25, 32, 38		10	

Complications & emergency management	8, 15, 16, 22, 33, 36		6	
Post-care, consent & documentation	7, 27, 29, 39, 40		5	

Final result category

Select	Result	Rule
[]	Pass with Distinction	Core score 90% or above and Advanced score at least 4/5.
[]	Pass	Core score 80% or above but distinction criteria not met.
[]	Retest Required	Core score 70-79%; targeted review and alternate-version retest.
[]	Retraining Required + Retest	Core score below 70%; retraining before complete retest.
[]	Invalid Attempt	Incomplete paper, misconduct or examiner-cancelled attempt.

Strong areas:

Areas requiring review:

Examiner comments:

Candidate signature	_____	Date	_____
Examiner signature	_____	Date	_____
Training Lead / Medical Director	_____	Date	_____

Remediation and retest record

Complete this page only when the result is Retest Required, Retraining Required + Retest, or an Invalid Attempt requiring a supervised repeat.

Original score		Original result	
Original version	A	Original date	
Required retest version		Retest date	

Areas reviewed and remediation completed

Competency area	Issue identified	Review / action completed
Scientific foundation & mechanism		
Indications & patient selection		
Contraindications & precautions		
Treatment protocol & technique		
Complications & emergency management		
Post-care, consent & documentation		

☐ Relevant slides reviewed ☐ Complete presentation repeated ☐ Manual reviewed

☐ One-to-one trainer discussion ☐ Case discussion ☐ Practical demonstration repeated

Trainer comments:

Retest outcome

Retest MCQ /80		Written /20		Core /100	
Advanced /5		Retest version		Final result	

Final decision / focused improvement plan:

Candidate signature		Date	
Examiner signature		Date	
Training Lead / Medical Director		Date	